Treatment scope · NCCP 00259 Lung monotherapy context only. The breast guide and its evidence remain separate. Discuss the exact disease setting and alternatives with the treating team.

NCCP 00259 · VERSION 6

Vinorelbine capsules · non-small-cell lung cancer

ONCE WEEKLY. First 3 administrations 60 mg/m², total maximum 120 mg. From administration 4, 80 mg/m² (maximum 160 mg) only after the team reviews nadirs and toxicity. 60 or 50 mg/m²/ lower may be needed. Continue weekly while helpful and tolerated; no fixed total number of weeks.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Milestones are not equal time intervals. Your team books and reviews tests. Blood counts are checked on the day of each weekly dose. Kidney and liver tests follow the weekly regimen and prescribed plan. The team reviews the lowest counts after earlier doses before any increase, alongside bowel, nerve, infection and medicine checks.

Check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Blood / kidney / liver	Baseline blood and organ tests	Counts before every weekly dose	Weekly treatment review	Review recovery and current dose	Follow unresolved abnormalities
Symptoms / medicines	Bowel / nerve / infection review	Report early symptoms and support problems	Review all medicines and side effects	Do not restart or escalate yourself	Agree continuation and response assessment
Special schedule check	Confirm weekly capsule plan	Review nadirs after doses 1–3	Dose increases only after team review	Keep previous dose / nadir history	Personal plan; no booked dates implied

THE WHOLE TREATMENT JOURNEY

Vinorelbine capsules · non-small-cell lung cancer: treatment and checks

Non-small-cell lung cancer · weekly oral monotherapy

ONCE WEEKLY. First 3 administrations 60 mg/m², total maximum 120 mg. From administration 4, 80 mg/m² (maximum 160 mg) only after the team reviews nadirs and toxicity. 60 or 50 mg/m²/lower may be needed. Continue weekly while helpful and tolerated; no fixed total number of weeks.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Treatment plan NCCP 00259	Confirm lung monotherapy plan and current dose	Weekly doses 1–3; review before dose 4	Continue while helpful and tolerated	Review dose and recovery before restart	Agree continuation / next treatment
Blood / kidney / liver NCCP 00259 · SMPC	Baseline blood and organ tests	Counts before every weekly dose	Weekly treatment review	Review recovery and current dose	Follow unresolved abnormalities
Symptoms / medicines NCCP 00259 · SMPC	Bowel / nerve / infection review	Report early symptoms and support problems	Review all medicines and side effects	Do not restart or escalate yourself	Agree continuation and response assessment
Special schedule check NCCP 00259 · SMPC	Confirm weekly capsule plan	Review nadirs after doses 1–3	Dose increases only after team review	Keep previous dose / nadir history	Personal plan; no booked dates implied

What each check is for

1 Treatment and dose review

Before every administration

Blood counts are checked on the day of each weekly dose. Kidney and liver tests follow the weekly regimen and prescribed plan. The team reviews the lowest counts after earlier doses before any increase, alongside bowel, nerve, infection and medicine checks.

Ask / act: The treating team confirms results, dose and suitability; this map does not authorise treatment.

Sources: NCCP 00259 · SMPC

2 Support and symptoms

Throughout treatment, including days at home

Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

Ask / act: Keep a written medicine plan and urgent-contact route.

Sources: NCCP 00259 · ORAL-ANTIEMETIC · NCIS

3 Response and continuation

At agreed clinical review points

Vinorelbine chemotherapy can help control non-small-cell lung cancer and its symptoms. The team weighs possible benefit against side effects, previous treatments and your priorities, and reviews whether to continue.

Ask / act: Confirm the current plan with the team.

Sources: NCCP 00259 · TRIAL

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.25 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending